

Patient NAME : Mrs Vidhi Kant Gupta
DOB/Age/Gender : 33 Y 8 M 13 D/Female
Patient ID / UHID : 17991883/RCL17698071
Referred BY : Self
Sample Collected : Jul 28, 2026, 09:28 AM

Report STATUS : Final Report
Barcode NO : 34280484
Sample Type : FLUORIDE F
Report Date : Jul 28, 2026, 03:49 PM.



Test Description	Value(s)	Unit(s)	Reference Range
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Muditam Full Body With Vitamin D and B12

Blood Sugar Fasting

Glucose Fasting Hexokinase	74.4	mg/dL	70 - 100
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Interpretation:

Status	Fasting plasma glucose in mg/dL
Normal	70 - 100
Impaired fasting glucose	101 - 125
Diabetes	≥ 126

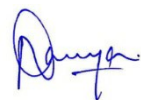
Reference : American Diabetes Association

Comment :

Blood glucose determinations are commonly used as an aid in the diagnosis and treatment of diabetes. Elevated glucose levels (hyperglycemia) may also occur with pancreatic neoplasm, hyperthyroidism, and adrenal cortical hyper function as well as other disorders. Decreased glucose levels (hypoglycemia) may result from excessive insulin therapy, insulinoma, or various liver diseases.

Note

1. The diagnosis of Diabetes requires a fasting plasma glucose of ≥ 126 mg/dL or a random / 2 hour plasma glucose value of ≥ 200 mg/dL with symptoms of diabetes mellitus.
2. Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis.



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Liver Function Test (LFT)

Bilirubin Total <i>Diazo</i>	0.496	mg/dL	0 - 1.2
Bilirubin Direct * <i>Diazo Reaction</i>	0.19	mg/dL	0.0 - 0.5
Bilirubin Indirect * <i>Calculated</i>	0.31	mg/dL	0.1 - 1.0
SGOT/AST <i>IFCC without P5P</i>	16.4	U/L	up to 32
SGPT/ALT <i>IFCC without P5P</i>	8.4	U/L	up to 33
SGOT/SGPT Ratio * <i>Calculated</i>	1.95 H*	Ratio	<1.00
Alkaline Phosphatase	61.8	U/L	35-104
Total Protein <i>Biuret</i>	7.6	g/dL	6.4 - 8.3
Albumin <i>BCG Colorimetric</i>	4.5	g/dL	3.5 - 5.2
Globulin * <i>Calculated</i>	3.1	g/dL	2.3 - 3.5
Albumin :Globulin Ratio * <i>Calculation (Albumin/Globulin)</i>	1.45	-	1.0 - 2.1
Gamma Glutamyl Transferase (GGT) * <i>Enzymatic Colorimetry</i>	9.8	U/L	5 - 36

Interpretation:

The liver filters blood, metabolizes nutrients, detoxifies harmful substances, and produces blood clotting proteins. Liver cells contain enzymes that facilitate these functions. When cells are damaged, enzymes leak into the blood, detectable through blood tests.

Key enzymes tested:

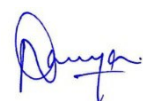
- AST (SGOT):** may indicate tissue injury / damage in muscles or liver.
- ALT (SGPT):** Primarily in the liver. Elevated ALT and AST suggest liver damage.
- Alkaline Phosphatase & GGT:** Linked to bile production and flow. Elevated levels may indicate bile flow issues related to the liver, gallbladder, or bile ducts.

Blood proteins, **albumin and globulin**, are essential for growth, development, and health.

- Low protein:** May indicate bleeding, liver disorders, malnutrition, or agammaglobulinemia.
- High protein (Hyperproteinemia):** Often due to dehydration or increased protein production.
- Low albumin:** Caused by poor diet, kidney, or liver disease.
- High albumin:** Usually due to severe dehydration.

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)

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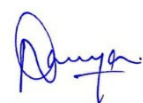
Kidney Function Test (KFT)

Blood Urea <i>Urease with UV</i>	24.2	mg/dL	16.6 - 48.5
Bun * <i>Urease</i>	11.31	mg/dL	6 - 20
Creatinine <i>Kinetic Alkaline Picrate</i>	0.7	mg/dL	0.5 - 1.2
eGFR (CKD-EPI)	116.52	ml/min/1.73 sq m	Normal Or High: ≥ 90 Mild Or Decrease: 60-89 Mild To Moderate Decrease: 45-59 Mild To Severe Decrease: 30-44 Severe Decrease: 15-29 Kidney Failure: < 15
Bun/Creatinine Ratio * <i>Calculated</i>	16.16	Ratio	12 - 20
Urea / Creatinine Ratio * <i>Calculated</i>	34.57	Ratio	25.68- 42.8
Uric Acid <i>Uricase</i>	3	mg/dL	2.4 - 5.7
Calcium Serum <i>BAPTA</i>	9.4	mg/dL	8.6 - 10.0
Phosphorus <i>Molybdate UV</i>	3.9	mg/dL	2.5 - 4.5
Sodium <i>ISE-Indirect</i>	139.5	mmol/L	136 - 145
Potassium <i>ISE-Indirect</i>	4.1	mmol/L	3.5 - 5.1
Chloride <i>ISE-Indirect</i>	102.9	mmol/L	98 - 107

Interpretation:

Kidney function tests is a collective term for a variety of individual tests and procedures that can be done to evaluate how well the kidneys are functioning. Many conditions can affect the ability of the kidneys to carry out their vital functions. Some lead to a rapid (acute) decline in kidney function others lead to a gradual (chronic) decline in function. Both result in a buildup of toxic waste substances done on urine samples, as well as on blood samples. A number of symptoms may indicate a problem with your kidneys. These include : high blood pressure, blood in urine, frequent urges to urinate, difficulty beginning urination, painful urination, swelling in the hands and feet due to a buildup of fluids in the body. A single symptom may not mean something serious. However, when occurring simultaneously, these symptoms suggest that your kidneys are not working properly. Kidney function tests can help determine the reason. Ionized calcium this test if you have signs of kidney or parathyroid disease. The test may also be done to monitor progress and treatment of these diseases. "eGFR test is applicable for patients aged 18 years or more."

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Lipid Profile

Total Cholesterol <i>Enzymatic - Cholesterol Oxidase</i>	199	mg/dL	<200
Triglycerides <i>Colorimetric - Lip/Glycerol Kinase</i>	111	mg/dL	<150
HDL Cholesterol <i>Accelerator Selective Detergent</i>	49.3	mg/dL	>40
Non HDL Cholesterol * <i>Calculated</i>	149.7 H*	mg/dL	<130
LDL Cholesterol * <i>Calculated</i>	127.5 H*	mg/dL	<100
V.L.D.L Cholesterol * <i>Calculated</i>	22.2	mg/dL	< 30
Chol/HDL Ratio * <i>Calculated</i>	4.04	Ratio	0.0 - 5.0
HDL/ LDL Ratio * <i>Calculated</i>	0.39 L*	Ratio	0.5 - 3.0
LDL/HDL Ratio * <i>Calculated</i>	2.59	Ratio	0.0- 3.0

Interpretation:

Lipid level assessments must be made following 10 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. NCEP recommends of 3 different samples to be drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays. Intraindividual (within-person) variation in triglyceride (TG) levels is high, often showing a 12.9% to 40.8% variation in healthy individuals within 1 year.. This variability is driven by a combination of biological, lifestyle, and physiological factors that fluctuate rather than remaining constant.

Causes of variation include

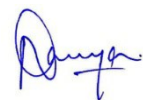
- Diet: Levels spike 5–10 times after meals. Unhealthy high saturated fat diets like non veg diet sweetened beverages , fried or processed foods , and alcohol intake can significantly raise triglyceride values.
- Lifestyle: Obesity , sedentary lifestyle, and smoking can raise levels.
- Biology: Pregnancy (estrogen), aging, and conditions like Diabetes or Metabolic Syndrome cause major shifts.
- Drugs like : Beta-blockers, steroids, tamoxifen, thiazides, and oral contraceptives can raise triglyceride levels

National Lipid Association Recommendations (NLA-2014)	Total Cholesterol (mg/dL)	Triglyceride (mg/dL)	LDL Cholesterol (mg/dL)	Non HDL Cholesterol (mg/dL)
Optimal	<200	<150	<100	<130
Above Optimal			100-129	130 - 159
Borderline High	200-239	150-199	130-159	160 - 189
High	>=240	200-499	160-189	190 - 219
Very High	-	>=500	>=190	>=220

HDL Cholesterol
Low
High

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	<40		>=60

Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.

Risk Category	A. CAD with > 1 feature of high risk group
Extreme risk group	B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease
Very High Risk	1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidence of end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >= 50 mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age >=45 years in Males & >= 55 years in Females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal <OR = 30)	<80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	>OR = 30	>OR = 60	> 30	> 60
Very High Risk	<50	<80	>OR = 50	>OR = 80
High Risk	<70	<100	>OR = 70	>OR = 100
Moderate Risk	<100	<130	>OR = 100	>OR = 130
Low Risk	<100	<130	>OR = 130*	>OR = 160

* After an adequate non-pharmacological intervention for at least 3 months.

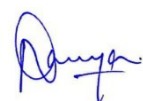
References :

- Management of Dyslipidaemia for the Prevention of Stroke : Clinical practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology,2022,20,134-155.
- Nordestgaard, B. A Test in Context: Lipid Profile, Fasting Versus Nonfasting. JACC. 2017 Sep, 70 (13) 1637–1646.
- P.N.M. Demacker, R.W.B. Schade, R.T.P. Jansen, A. Van 't Laar, Intra-individual variation of serum cholesterol, triglycerides and high density lipoprotein cholesterol in normal humans, Atherosclerosis,
- Parhofer KG, Laufs U: The diagnosis and treatment of hypertriglyceridemia. Dtsch Arztebl Int 2019; 116: 825–32. DOI: 10.3238/arztebl.2019.0825

*** End Of Report ***

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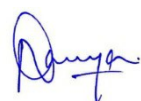


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Pending Report To Follow - Vitamin D, TFT TOTAL, HbA1C, ESR, CBC, Vitamin B12

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2. All the test results are based strictly on the specimen/sample and information received under the details provided at the time of registration. The patient/customer or referring entity is solely responsible for the accuracy and completeness of details furnished before sample submission. The laboratory shall not be liable for errors, misidentification, or incorrect results arising due to inaccurate, incomplete, or misleading information provided at the time of registration.
3. The patient/customers must disclose all relevant conditions including but not limited to fasting status, medications, dietary intake, alcohol consumption, lifestyle factors, ongoing treatments, and medical history that may affect test outcomes. The laboratory shall not be responsible for any deviation in results due to non-disclosure or incorrect disclosure of such information.
4. Test results are dependent on the quality, integrity, and timing of the specimen received. The laboratory reserves the right to reject or request repeat samples in cases of inadequate, hemolyzed, contaminated, or otherwise compromised samples. No liability shall arise for inability to process or delay due to such reasons.
5. Test results are subject to inherent biological variation and analytical variation depending on the nature of the test, methodology, instrumentation, and pre-analytical factors. While the laboratory follows established quality control and assurance standards, minor variations in results may occur and should be interpreted in conjunction with clinical findings, patient history, and other diagnostic information. The laboratory does not guarantee absolute accuracy, precision, or reproducibility of results across different testing instances or laboratories, and no liability shall arise on account of such inherent variations.
6. All turnaround times are indicative, not guaranteed. Delays may occur due to test complexity, sample condition, referral testing, or quality control procedures, logistics, or other operational constraints.
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11. In case of critical, alarming, or unexpected test results, you are advised to contact the laboratory immediately for further guidance and thereafter consult a qualified medical practitioner without delay. Test results should always be interpreted in conjunction with the patient's clinical history and clinical findings.
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